4 Basic Life Support (Adult)

HME922 Procedural Station · 10 minutes · HB6 BLS & First Aid lecture + ANZCOR 2024

guideline wording — say it aloud DOPS On the examiner's DOPS marking form ☐ tick when done

KEY MESSAGE

Unresponsive and **not breathing normally** → follow the **ANZCOR BLS flowchart: DRSABCD**. Say every letter out loud. **30:2 at 100–120/min, 1/3 depth.** AED on **ASAP**.

UNIVERSAL OPENING (EMERGENCY VERSION)

- ☐ Introduce yourself to the examiner: name + "medical student"
- ☐ State: "Emergency — **consent is implied**"
- ☐ Apply **PPE — protective eyewear + gloves** before touching the patient
- ☐ Go straight to DRSABCD; talk the examiner through each step
- ☐ Patient wakes → introduce yourself, explain, reassure

MEMORY AID — DRSABCD — "Doctors' Rounds Start At Breakfast, Coffee 'n' Danish"

Danger · **R**esponse · **S**end for help · **A**irway · **B**reathing · **C**PR · **D**efibrillation

learning objectives: purpose of BLS · technical **and communication** skills for DRSABCD · **routine post-resuscitation care**

D — DANGER

- ☐ First priority: scene safe for **1 rescuer** (PPE: eyewear + gloves) · **2 patient** · **3 others** helping

Community dangers	Healthcare dangers
Violence · fire, smoke, fumes · traffic	Blood + body fluids · needles/sharps
Confined spaces · unstable structures	Wet floors / spilled fluids
Electricity · sharps · dangerous weather	**Electrical equipment** — don't touch a patient still connected to current
Deep/fast water · chemicals · biological	**Trip hazards** — move furniture/people to make space

R — RESPONSE ("TALK AND TOUCH")

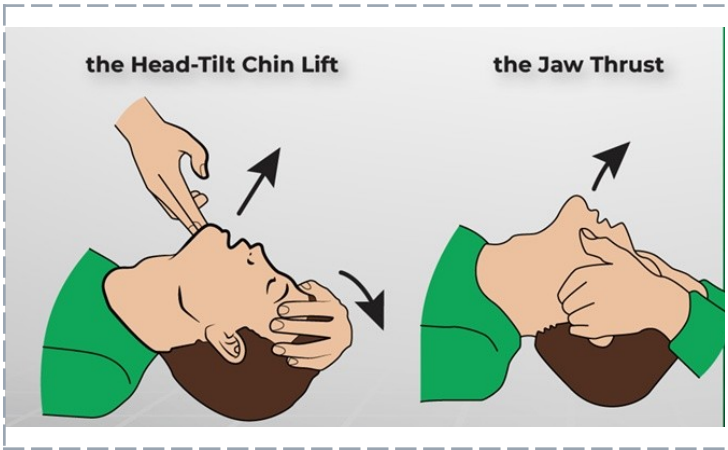
1. **Verbal:** "Can you open your eyes?"
2. **Touch:** "Can you squeeze my hand?"
3. **Painful:** firmly squeeze the shoulders (**trapezius**)

S — SEND FOR HELP

- ☐ **Delegate someone to get the AED**
- ☐ **In hospital:** call out — "**Hello, can I get some help in here please!**" · press emergency bell · dial **2222** (standard emergency number) → your **name, location (ward/bed)**, "**Code Blue**"
- ☐ **Out of hospital: 000**

A — AIRWAY (POSITION → OPEN & OBSERVE → OPTIMISE)

- ☐ Airway takes priority over **any** other injury, **including possible spinal injury** (ANZCOR 9.16)
- ☐ **Position:** adult + child = **sniffing** · infant = **neutral**
- ☐ **Open & observe:** can you see anything? Remove without pushing it further down — suction or head position. **No finger sweeps**
- ☐ **Optimise:** head tilt–chin lift or jaw thrust · oral airway adjunct (OPA) · suction



B — BREATHING

- ☐ Keep the airway open (manoeuvre / adjunct) **while** you check
- ☐ **Look** for chest movement · **Listen** for breath sounds · **Feel** breath on your hand
- ☐ Check for **no more than 10 seconds**; gasping/agonal breathing = **not normal**
- ☐ Unresponsive + not breathing normally → **start compressions immediately**

C — CPR

	Adult	Child	Infant
Hands	Two hands, lower half of sternum	One or two hands	Two fingers or encircling thumbs
Depth	**1/3** of chest	**1/3**	**1/3**
Rate	**100–120 / min** (~2 per second)	same	same
Ratio	**30 : 2**	30 : 2	30 : 2
Breaths	**1 second each**; mouth-to-mask or BVM; see chest rise		

- ☐ Rates **< 100 or > 140** are linked to lower survival (ANZCOR Guideline 6)
- ☐ Full recoil every compression; pauses < 10 s; swap compressor ~every 2 min

> **MEMORY AID — NUMBERS — "30 – 2 – 100–120 – 1/3 – 2 min"**
> 30 pushes, 2 breaths, 100–120 a minute, one-third deep, AED re-analyses every 2 minutes.
> Beat: "**Stayin' Alive**" ≈ 103 bpm.

D — DEFIBRILLATION (AED)

1. Only for someone **unresponsive and not breathing normally**
2. Attach **ASAP**, turn **on**, follow voice prompts
3. Remove clothing; attach pads (upper right chest + left side). **Pads touching?** → one middle of chest, one on back between shoulder blades
4. Plug pad cable into the AED
5. **Analysis: nobody touches the patient**

AED says	You do
Shockable	Charges (150 or 200 J) → **visual check** + loudly "**STAND CLEAR!**" → press shock → **CPR 2 minutes** with no interruptions
Non-shockable	Won't charge → "resume compressions" → CPR 2 minutes
Every 2 minutes	AED re-analyses — hands off

- ☐ Pads **not** over jewellery, pacemakers or medication patches
- ☐ Clip excess chest hair; wipe off moisture; pads firmly stuck
- ☐ Child **< 8 years** (incl. infants): paediatric attenuated pads (**50 J**) — or standard adult pads if none

> **MEMORY AID — SHOCK SEQUENCE — "Look, Shout, Shock, Push"**
> **Look** (visual check nobody touching) → **Shout** "Stand clear!" → **Shock** → **Push** (straight back to compressions)

CONTINUE BLS UNTIL

- ☐ **Signs of life** return
- ☐ **Qualified help** arrives and takes over
- ☐ **Impossible to continue** (e.g. exhaustion)
- ☐ An **authorised person pronounces death**

POST-RESUSCITATION CARE

- ☐ **Explain** what happened to the patient **and family**
- ☐ **Recovery position — left lateral**
- ☐ **Oxygen** if available
- ☐ Monitor closely: **reassess ABCDE** + vital signs (**HR, RR, BP**)
- ☐ To **ED** for observation + investigations (bloods, **ECG**, imaging) — **find the cause**, prevent recurrence
- ☐ Keep watching for further deterioration

CHOKING (AIRWAY GUIDELINE)

Severity	Action
Mild — effective cough	Sit up, encourage coughing, monitor, send for help
Severe — ineffective cough	Call for help; back blows then chest thrusts (up to 5 each), alternate; suction / forceps with caution
Becomes unresponsive	Start DRSABCD

UNIVERSAL CLOSING (EMERGENCY VERSION)

- ☐ **ISBAR handover** to Code Blue/ambulance: time of collapse, CPR start, number of shocks, return of circulation
- ☐ Sharps (if any) to sharps bin; waste sorted
- ☐ Recovery position (left lateral); oxygen; privacy; family informed
- ☐ Hand hygiene — Moments 4/5
- ☐ Document times, actions, shocks, outcome, name + signature
- ☐ Debrief with team; restock resus trolley / AED
- ☐ Examiner asks **"How do you think that went?"** → one thing done well · one to improve · one strategy

MY NOTES